



External Cephalic Version Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	Obstetrics	Nursing, Midwifery and Medical

2. Purpose

Performing an External Cephalic Version (ECV) has been shown to reduce the rate on non-cephalic presentations at term reducing the number of caesarean sections (CS) for the breech at term. Pregnant women who have breech presentation identified in the third trimester should be offered ECV, providing there are no contraindications.

Success rate for ECV is approximately 40% for nulliparous women and 60% for multiparous women. Few babies (2.2%) will revert to breech or spontaneous turn after 36 weeks (Level B evidence)

Risk of complications include:

Abnormal cardiotocograph (CTG) patterns (0.9%)

Antepartum haemorrhage (0.08%)

Cord complications

Spontaneous ruptured membranes

Fetal mortality.

Perinatal mortality rate (PNMR) for elective CS after 39w is 0.5/1000, while PNMR is 1/1000 for cephalic birth and 2/1000 for vaginal breech delivery: 4% may turn to spontaneous vertex position after failed ECV.

Planned CS will give a small reduction in perinatal mortality compared to vaginal breech delivery, but this should be balanced by the adverse consequences of the CS with a small increase in complications for the mother compared to vaginal birth. CS does increase the risk in subsequent pregnancies.

3. Contra-indications

There is limited evidence concerning contraindications to ECV. Following absolute and relative contraindications should be taken as a guide when contemplating on performing ECV:

3.1 Absolute

- Where CS is Indicated
- Antepartum Haemorrhage (APH)
- Severe preeclampsia
- Abnormal cardiotocograph (CTG)
- Abnormal doppler

- Major uterine anomalies
- Ruptured membranes
- Multiple pregnancy except after delivery of first twin
- Fetal hypoxia
- Fetal death
- Placental abruption
- Oligohydramnios
- Rhesus iso immunisation
- When mother has declined ECV or is unable to give consent

3.2 Relative

- IUGR
- Unstable lie before stabilising induction
- Scarred uterus after previous one CS. Woman should be informed that ECV after one CS delivery appears to have no greater risk than with an unscarred uterus.

4. Process

4.1 Breech at 36 weeks period of gestation (POG) onwards in antenatal clinic:

- Ensure the woman has received counselling about risks, benefits, and outcomes associated with performing ECV. Patient information leaflet and consent form will be given to patient to read and sign at home and bring on day of procedure.
- The women should be referred for ECV after 36 completed weeks onwards. Nulliparous women can be referred for ECV from 36 weeks onwards. An appointment needs to be booked in the AAU with information of date and time given to the woman
- The appointment should only be booked with a Specialist Obstetrician who is trained and is willing to perform an ECV. GP Obstetricians can perform the procedure under strict supervision of the consultant available or on call consultant
- A formal ultrasound must have been performed within a reasonable timeframe (1-7 days) to assess fetal presentation, placental location, amniotic fluid volume and assessment of fetal or uterine anomalies.
- Check there are no contraindications to performing ECV.
- Keep the woman informed of possible delays or postponement of the procedure if the obstetrician or labour ward is busy.

5. Procedure

5.1 On arrival to AAU:

- Perform baseline maternal observations of pulse, respiration rate, blood pressure, urinalysis and abdominal palpation.
- Check the woman's blood group. Rh negative women will require anti-D
- Perform CTG for 20 minutes prior to the procedure
- Confirm consent is signed for ECV and in the file.
- Confirm breech presentation by a portable ultrasound machine before starting the procedure

- Arrange written orders for subcutaneous Terbutaline 0.25mg (250mcg) or Nifedipine, according to specialist preference.
- Monitor maternal pulse, BP, and fetal heart rate (FHR) every 10 minutes after tocolysis is given until the ECV commences
- Before giving uterine relaxants ensure that the specialists performing the procedure is available within 20-30 minutes of giving the drug
- Ensure OT is available if emergency LSCS is required; however, the standard pre-operative preparations for CS are not recommended.
- Ensure the woman has emptied her bladder.

5.2 Abandon the procedure

- If three attempts of ECV, each lasting 3 minutes are unsuccessful, discuss further management plans with the patient. No more than four attempts are advised for a suggested maximum time of 10 minutes. 5
- Maternal intolerance to the procedure
- Evidence of abnormal FHR using sonography
- Ultrasound may be used during and after ECV to check FHR.

5.3 Post procedure

- Following procedure patient should be placed upright in the bed
- CTG should be performed for 40 minutes whether ECV is successful or not
- A transient (less than 3 min) bradycardia is common after ECV, but should instigate strict fetal monitoring in left lateral position and if persistent after 6 minutes should prompt preparations for cat 1 CS. 5
- A reactive CTG will be obtained prior to discharge
- Monitor and record maternal pulse, BP, and any vaginal loss every 15 minutes for at least 30 minutes
- Women who are Rh negative should undergo testing for feto-maternal haemorrhage (risk is 2.4%), to decide if these women might benefit from an additional dose of anti-D. A strongly positive (>30ml) test should prompt urgent fetal review. Blood results must be followed up as per AAU protocol of checking pending results.
- The woman can be discharged after one hour providing maternal observations are normal and:
 - The CTG is reactive
 - The specialist is satisfied with the fetal and maternal condition
- Instruct the woman to return if any of the following occur:
 - Vaginal bleeding
 - Rupture of Membranes
 - Labour commences
 - Decrease in fetal movements
 - Abnormal abdominal pain
- Ensure a follow-up appointment with the Doctor or Midwife one week later to confirm the presenting part has remained in the cephalic position.

6. Legislative context

The following documents are required to give effect to this procedure:

- Health Department of Western Australia (HDWA) Consent to Treatment Policy OD 0657/16
- HDWA Clinical Handover MP 0095
- East Metropolitan Health Service (EMHS) Patient Identification and Procedure Matching EMHS: 21

7. Supporting information

The following documents support the implementation of this procedure:

- Breech Presentation: Vaginal Birth Guideline OBS-GUI-0066-11

8. Compliance, monitoring and evaluation

Compliance against this procedure will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS.

Specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunities for improvement.

9. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.7 Planning for comprehensive care
- The health service organisation has processes relevant to the patients using the service and the services provided:
 - a. For integrated and timely screening and assessment
 - b. That identify the risks of harm in the 'Minimising patient harm' criterion
- Standard 6 – Action 6.4 Organisational processes to support effective communication
- The health service organisation has clinical communications processes to support effective communication when:
 - a. Identification and procedure matching should occur
 - b. All or part of a patient's care is transferred within the organisation, between multidisciplinary teams, between clinicians or between organisations; and on discharge
 - c. Critical information about a patient's care, including information on risks, emerges or changes
- Standard 6 – Action 6.5 Correct identification and procedure matching
- The health service organisation:
 - a. Defines approved identifiers for patients according to best-practice guidelines
 - b. Requires at least three approved identifiers on registration and admission; when care, medication, therapy and other services are provided; and when clinical handover, transfer or discharge documentation is generated
- Standard 6 – Action 6.7 Clinical handover
- The health service organisation, in collaboration with clinicians, defines the:

- a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
- b. Risks relevant to the service context and the particular needs of patients, carers and families
- c. Clinicians who are involved in the clinical handover
- Standard 6 – Action 6.9 Communication critical information
- Clinicians and multidisciplinary teams use clinical communication processes to effectively
- communicate critical information, alerts and risks, in a timely way, when they emerge or change to:
 - a. Clinicians who can make decisions about care
 - b. Patients, carers and families, in accordance with the wishes of the patient

10. References

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7. Royal Australian and New Zealand College of Obstetricians college of Obstetricians and Gynaecologists. Management of the Term Breech Position. 2013 RANZCOG College Statement C-Obs 11. <https://www.ranzcog.edu.au/college-statements-guidelines.html#obstetrics>.
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10. Rosman AN, Guijt A,Vlemmix F,Rijnders M, Mol BW, Kok M. Contra indications for ECV in breech position at term: a systematic review. *Acta Obstet Gynaecol Scand* 2013; 92:137-142
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12. Boucher M,Marquette GP, Varin J, Champagne J, Bujold E. Fetomaternal hemorrhage during external cephalic version. *Obstet Gynaecol* 2008;112:79-84.

11. Policy contact (relating to these procedures)

Enquiries relating to this procedure may be directed to:

Title: HOD, Obstetrics & Gynaecology
 Department: Maternity
 Email: sangeeta.mallabhat@health.wa.gov.au

12. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter. Evidence reviewed and overall guidance remains unchanged. Minor changes have been made

Version	Effective from	Effective to	Amendment(s)
V3	28/09/2023	28/09/2026	Scheduled review

13. Authorisation

This procedure has been authorised and issued by the Director Clinical Services.

Authorised by	Dr Alison Parr – Director Clinical Services
Authorisation date	22/09/2023
Published date	28/09/2023